



yes please

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Global Procedure Codes and Global Days – Quick Reference

1. Global Day Indicators (CMS)

- **000 – 0-Day Global**
 - No pre-op or post-op period.
 - Typical for:
 - Minor procedures
 - Many endoscopies
 - Some diagnostic/therapeutic procedures
- **010 – 10-Day Global**
 - Surgery day + 10 post-op days (11 total).
 - Includes:
 - Procedure
 - Typical post-op visits related to that procedure
- **090 – 90-Day Global**
 - 1 pre-op day + surgery day + 90 post-op days (92 total).
 - Major surgeries.
- **045 – 45-Day Global**
 - Rare category; used for specific maternity/postpartum scenarios.
- **XXX / 999 – Global Concept Does Not Apply**
 - No global period.
 - Typically:
 - E/M services
 - Labs

- Diagnostic tests
- Many radiology and medicine codes

2. Examples by Specialty / Body System

Note: Exact global days are payer- and year-specific. Always verify against the current CMS MPFS or payer policy.

2.1 Integumentary (10000–19999)

Common 10-day global examples

- 10060 – Incision and drainage of abscess; simple
- 10061 – Incision and drainage of abscess; complicated
- 11400–11446 – Benign lesion excision, various sizes/sites
- 12020–12021 – Simple wound exploration and repair
- 17000 – Destruction (e.g., premalignant lesion, first lesion)

Common 90-day global examples

- 11450–11471 – Excision of malignant lesions (by size/site)
- 13160 – Secondary closure of surgical wound or dehiscence
- 14000+ – Adjacent tissue transfer/rearrangement (local flaps)
- 15050 – Surgical preparation of recipient site (e.g., skin graft)

2.2 Musculoskeletal / Orthopedic (20000–29999)

Typically 90-day global

- 20661 – Injection of joint with ultrasound guidance (check payer; may vary)
- 21010 – Excision of benign lesion of mandible
- 23000 – Incision and drainage of shoulder area
- 27130 – Total hip arthroplasty
- 27447 – Total knee arthroplasty

(Some minor orthopedic procedures may carry 0- or 10-day globals; verify each code.)

2.3 Ophthalmology (65000–68999)

Common 90-day global example

- 66984 – Extracapsular cataract removal with IOL (routine cataract)

Minor ophthalmic procedures can be 0- or 10-day global depending on the code.

2.4 GI / Endoscopy (40000–49999, selected)

Many endoscopies and minor GI procedures:

- Commonly **0-day** global (000) – no post-op period; each encounter distinct.

Major abdominal surgeries:

- Typically **90-day** global (090).

Examples to verify in MPFS (global likely 0 or 90 depending on complexity):

- 43235 – Upper GI endoscopy; diagnostic
- 45378 – Colonoscopy; diagnostic
- 44120+ – Small intestine resection (major; often 90 days)

2.5 OB / Maternity (59000–59999)

Some OB codes historically associated with **45-day** global periods (payer-specific; now often handled via OB package/bundles), e.g.:

- 59409 – Vaginal delivery only
- 59410 – Vaginal delivery including postpartum care
- 59514 – Cesarean delivery only
- 59515 – Cesarean delivery including postpartum care
- 59618–59622 – VBAC/C-section with/without postpartum care
- 59430 – Postpartum care only

Always check current payer OB/global policies for exact handling.

2.6 E/M and “No Global” Services (Most 99202–99499, Labs, Imaging)

- Office/outpatient visits (99202–99215): **no surgical global** – “XXX/999”.
- Labs (80047+, 81000+, 87003+): **no global**.
- Many imaging codes (70010+, 93000+): **no global**, though technical/professional components may apply.

3. Practical Coding Notes

- Use **Modifier 57** when the E/M service results in a decision for a **major surgery (90-day global)** on the same or following day.
- Use **Modifier 25** for a **significant, separately identifiable E/M** on the same day as a minor procedure (0- or 10-day global).
- Use **Modifiers 54, 55, 56** to split the global care between:
 - 54 – Surgical care only

- 55 – Postoperative management only
- 56 – Preoperative management only

4. How to Turn This into a PDF

1. Open **Word, Google Docs**, or similar.
2. Paste this entire text.
3. Add any payer-specific notes you use (e.g., BCBSNC, Medicare JH/JM, etc.).
4. Export:
 - In Word: **File** → **Save As** → **PDF**
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If you tell me which payers you work with most (e.g., Medicare JH/JM, BCBS NC, UHC, etc.), I can extend this content with payer-specific global quirks or a more detailed appendix table you can drop right into your document.